

Policy on Infection Prevention and Control Surveillance

«Hospital Name»

Document control

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Prepared by	Infection Prevention and Control Officer Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

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Statement of intent

«Hospital Name»'s IPC surveillance programme tracks infection risks, rates and trends using a mix of active and passive surveillance; directs surveillance at identified high-risk activities; monitors hand-hygiene compliance at least monthly; captures MDROs; monitors housekeeping effectiveness; provides regular feedback to healthcare providers; identifies and responds to outbreaks; and analyses surveillance data for corrective and preventive action.

1. Purpose

This policy explains how «Hospital Name» performs IPC surveillance to capture and monitor infection risks, rates and trends, verify data, direct surveillance at high-risk activities and areas, monitor hand-hygiene compliance, capture multi-drug-resistant organisms, monitor housekeeping effectiveness, feed back surveillance data to healthcare providers, identify and respond to outbreaks, and take corrective and preventive action based on surveillance findings.

The clinical IPC processes, HAI-prevention bundles, and IPC programme governance are covered in separate hospital policies.

2. Scope

This policy applies to the IPC Officer, IPC Nurse(s), link nurses, the IPC committee, and all clinical staff whose practice is assessed through IPC surveillance activities at «Hospital Name».

3. Policy standards

«Hospital Name» uses a judicious mix of active and passive surveillance to track infection risks, rates and trends on an ongoing basis, calculating risk-adjusted rates (not only numerator counts), and takes suitable steps based on analysis. Surveillance data is authenticated by the IPC team. Periodic surveillance is conducted in all identified high-risk activities and areas. Hand-hygiene compliance is monitored at least monthly across all categories of direct-patient-care staff, with results shared with relevant staff. The occurrence and spread of MDROs is monitored. The effectiveness of housekeeping is monitored regularly using outcome parameters — not only frequency verification; routine microbiological environmental sampling is not used as a monitoring method; periodic fumigation is not conducted; fogging is used only in the three specified conditions. Surveillance feedback is provided regularly to the appropriate healthcare provider, covering adherence rates, HAI rates, trends and improvement opportunities. Baseline rates are established; written outbreak guidance is implemented. The IPC committee analyses data and takes CAPA where necessary.

Staff follow the written guidance below and keep the records it requires.

4. Non-negotiable rules

- Do not conduct surveillance that captures only numerator counts — calculate risk-adjusted rates using both numerator and denominator.
- Do not monitor hand-hygiene compliance less often than once a month, and do not exclude any category of direct-patient-care staff from monitoring.
- Do not use routine microbiological environmental sampling of OT and high-risk areas as a monitoring method.
- Do not conduct periodic fumigation.
- Do not fog except in the three specified conditions: commissioning a new OT; modification or renovation of an OT; significant increase in SSI with evidence of environmental involvement.
- Do not withhold surveillance feedback from the appropriate healthcare provider.
- Do not fail to establish baseline rates, and do not operate without written guidance for handling outbreaks.

5. What we do

5.1 Track infection risks, rates and trends

«Hospital Name» uses a judicious mix of active and passive surveillance. Surveillance data is collected on an ongoing basis at regular intervals and consolidated into reports. The organisation takes suitable steps based on analysis of this data. Surveillance goes beyond simple numerator counts — risk-adjusted rates are calculated using both numerator (infected patients) and denominator (all at-risk patients) data.

5.2 Verify surveillance data

Surveillance data is authenticated by the IPC team by reviewing every datum or by using random sampling. Where the IPC team collects all surveillance data itself, separate verification by a second party is not required.

5.3 Direct surveillance at identified high-risk activities

«Hospital Name» conducts periodic surveillance in each identified high-risk activity and high-risk area. The frequency and mode of surveillance are defined for each. The surveillance system adheres to national/international guidelines. Surveillance includes areas where demolition, construction or repairs are undertaken.

5.4 Monitor hand-hygiene compliance

Hand-hygiene compliance is monitored at a minimum once every month. An appropriate sample size is chosen and all categories of staff involved in direct patient care are monitored — no category is exempt. Compliance levels are shared with the relevant staff. The WHO Observation Form or an equivalent validated observation tool may be used.

5.5 Capture multi-drug-resistant organisms

«Hospital Name» monitors the occurrence and spread of multi-drug-resistant organisms (MDROs). Examples of organisms monitored include methicillin-resistant *Staphylococcus aureus* (MRSA), multi-drug-resistant gram-negative bacteria, and Vancomycin-resistant enterococci (VRE). The organisation's own antibiogram data may identify additional MDROs to track.

5.6 Monitor the effectiveness of housekeeping

The effectiveness of housekeeping services is monitored regularly. The organisation defines the periodicity of this monitoring. This applies even when housekeeping is outsourced. Effectiveness is captured through outcome parameters — such as cleanliness of premises, patient feedback, and staff feedback — not only by verifying that housekeeping was done at the defined frequency. **IMPORTANT:** Routine microbiological environmental sampling of OT and other high-risk areas is not used as a routine monitoring method. Periodic fumigation is not conducted. Fogging is used only when: commissioning a new OT; following modification or renovation of an OT; or when there is a significant increase in SSI with evidence of environmental involvement.

5.7 Feed surveillance data back to healthcare providers

Surveillance findings are fed back regularly to the appropriate healthcare provider — reaching unit in-charges, treating doctors, and others who can act on the data, not only the IPC office. Feedback includes adherence rates, HAI rates, trends, and opportunities for improvement, including data from other surveillance activities. Specific inputs to reduce HAI rates may also be included. Feedback may be in the form of a bulletin or newsletter.

5.8 Identify and respond to outbreaks

«Hospital Name» establishes baseline infection rates to define what constitutes an outbreak. Written guidance for identifying and handling outbreaks is implemented. The surveillance system is designed to enable early identification of outbreaks by detecting a rate above the established baseline. When an outbreak is identified, appropriate actions are taken to control it; corrective actions are implemented to prevent recurrence.

5.9 Analyse surveillance data and take corrective and preventive action

The IPC committee analyses surveillance data at each meeting. Corrective and preventive actions are taken where the analysis identifies the need. This includes actions following outbreaks. The IPC Officer tracks open CAPA items to closure.

6. Governance and responsibility

Role	Responsibility
Medical Superintendent	Accountable for ensuring the IPC surveillance programme is resourced and its findings acted upon.
Infection Prevention and Control Officer	Owns the IPC surveillance programme; ensures ongoing data collection, analysis and reporting; leads outbreak response; tracks CAPA to closure; brings surveillance data to the IPC committee.
Infection Prevention and Control Nurse(s)	Conducts active surveillance, hand-hygiene compliance monitoring, and housekeeping effectiveness monitoring; collates data for the IPC Officer.
IPC Committee	Analyses surveillance data at each monthly meeting; decides on CAPA; reviews outbreak response and corrective actions.
Department Heads	Ensure compliance with corrective actions arising from surveillance findings in their departments; disseminate feedback to relevant staff.
Quality Coordinator	Audits this policy; holds training records and staff acknowledgements.

7. Quality monitoring

Monitoring area	What is monitored
Surveillance scope and rates	Mix of active and passive surveillance in use; risk-adjusted rates calculated and reported; suitable steps taken on findings.
Data verification	Authentication by IPC team documented (or nil-verification note where IPC team collected all data).
High-risk area surveillance	Periodic surveillance records for each

	identified high-risk area and activity, with defined frequency and mode.
Hand-hygiene compliance	Monthly monitoring records covering all direct-patient-care staff categories; compliance levels shared with relevant staff.
MDRO monitoring	MDRO occurrence and spread tracked; organism log maintained.
Housekeeping effectiveness	Effectiveness monitored using outcome parameters; no routine environmental sampling; no periodic fumigation; fogging only in three specified conditions.
Feedback to providers	Regular feedback including adherence rates, HAI rates, trends and improvement opportunities reaching the appropriate healthcare provider.
Outbreak identification and response	Baseline rates established; written outbreak guidance implemented; outbreak response records and post-outbreak CAPA.
CAPA	IPC committee CAPA records with owners, due dates, and closure evidence.

8. Training and staff acknowledgement

The IPC Officer, IPC Nurse(s), and link nurses shall be familiar with the surveillance methods, monitoring tools, outbreak definition and response process, and the data feedback and CAPA cycle described in this policy.

I have read the Policy on Infection Prevention and Control Surveillance of «Hospital Name». I will follow the processes described.

Staff name	Designation	Signature	Date
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____

9. Distribution

This policy shall be available to the IPC Officer, IPC Nurse(s), the IPC committee, department heads, and the Quality Coordinator.

10. Abbreviations

Abbreviation	Meaning
CAPA	Corrective and Preventive Action
HAI	Healthcare Associated Infection
IPC	Infection Prevention and Control
IPCC	Infection Prevention and Control Committee
IPCO	Infection Prevention and Control Officer

IPCN	Infection Prevention and Control Nurse
MDRO	Multi-Drug-Resistant Organism
MRSA	Methicillin-Resistant Staphylococcus aureus
NABH	National Accreditation Board for Hospitals and Healthcare Providers
OT	Operation Theatre
SSI	Surgical Site Infection
VRE	Vancomycin-Resistant Enterococci
WHO	World Health Organization

11. Traceability table

This table is an index. It is not how the policy is organised. An asterisk in the Level column means documentation of the process is required.

Objective Element	Level	Traceability to this policy
IPC.6.a	CORE	Sections 3 and 5.1 address the use of active and passive surveillance, ongoing regular data collection, suitable steps based on analysis, and calculation of risk-adjusted rates.
IPC.6.b	Commitment	Sections 3 and 5.2 address data authentication by the IPC team through full review or random sampling, with the carve-out for IPC-team-collected data.
IPC.6.c	Commitment	Sections 3 and 5.3 address periodic surveillance directed at identified high-risk activities and areas, with defined frequency and mode, including construction/demolition areas.
IPC.6.d	CORE	Sections 3 and 5.4 address monthly hand-hygiene compliance monitoring of all direct-patient-care staff categories, with results shared with relevant staff.
IPC.6.e	Achievement	Section 5.5 addresses a mechanism to monitor the occurrence and spread of MDROs.
IPC.6.f	CORE	Sections 3 and 5.6 address regular monitoring of housekeeping effectiveness

		using outcome parameters; the prohibitions on routine environmental sampling, periodic fumigation, and non-conditional fogging.
IPC.6.g	Commitment	Sections 3 and 5.7 address regular feedback to the appropriate healthcare provider covering adherence rates, HAI rates, trends and improvement opportunities.
IPC.6.h	Commitment*	Sections 3 and 5.8 address establishment of baseline rates, written outbreak guidance, early identification through surveillance, and post-outbreak corrective action.
IPC.6.i	Commitment	Sections 3 and 5.9 address IPC committee analysis of surveillance data, CAPA where necessary, and post-outbreak corrective actions tracked to closure.

12. Required Records/Evidence Checklist

Infection risk, rate and trend tracking — IPC.6.a (CORE)

- Surveillance reports showing ongoing data collection at regular intervals.
- Evidence that risk-adjusted rates (not only numerator counts) are calculated.
- Records of suitable steps (actions) taken in response to analysis findings.

Data verification — IPC.6.b

- Authentication records (sample-based or full review by IPC team), or a documented note that the IPC team collected all data (verification not required).

High-risk area surveillance — IPC.6.c

- Surveillance records for each identified high-risk area and activity, including construction/demolition areas.
- Defined frequency and mode of surveillance documented per high-risk area.

Hand-hygiene compliance monitoring — IPC.6.d (CORE)

- Monthly compliance monitoring records covering all categories of direct-patient-care staff.
- Evidence that compliance levels are shared with relevant staff (e.g., bulletin, feedback sheet, department communication).

MDRO monitoring — IPC.6.e (Achievement)

- MDRO occurrence and spread log, with records of organisms detected and containment measures taken.
- Evidence the MDRO capture mechanism is operational (microbiologist alert to IPC team, or equivalent).

Housekeeping effectiveness monitoring — IPC.6.f (CORE)

- Defined outcome parameters for housekeeping effectiveness monitoring.
- Regular monitoring records using those outcome parameters.
- Record confirming no routine microbiological environmental sampling is used as a monitoring method.
- Records confirming fogging was used only under the three permitted conditions (or a nil record if no fogging has occurred).

Feedback to healthcare providers — IPC.6.g

- Periodic feedback documents (bulletin, report, or equivalent) that include adherence rates, HAI rates, trends, and improvement opportunities.
- Evidence that feedback reached the appropriate healthcare provider — unit in-charges, treating doctors, or department heads.

Outbreak identification and response — IPC.6.h (Commitment*)

- Baseline infection rates established and documented.
- Written outbreak definition using those baseline rates.
- Written outbreak-handling guidance.
- Outbreak response records (or a drill record if no real event has occurred), including post-outbreak corrective action.

Surveillance data CAPA — IPC.6.i

- IPC committee minutes showing surveillance data analysis and CAPA decisions.
- Open-action tracking records showing owners, due dates, and evidence of closure.

13. References

1. National Accreditation Board for Hospitals and Healthcare Providers. NABH Accreditation Standards for Hospitals, 6th Edition. IPC.6.
2. World Health Organization. (n.d.). <https://www.who.int/teams/integrated-health-services/infection-prevention-control/hand-hygiene> WHO Multimodal Hand Hygiene Improvement Strategy and Observation Form for Hand Hygiene Compliance.

Disclaimer

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[Hospital to define] must be replaced with the hospital's own decision; a document issued with those markers left in place is not an approved policy.

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